

Bites

Animal Bites

- Type of animal? Human Bite?
- Is animal a pet or known to patient? Is animal a stray?
- Has animal been captured?
- If known, is animal up to date on all vaccinations?
- Area of body? – Neck, face, head, axilla, groin, abdomen are more dangerous areas.
- Is bleeding controlled?
- Does patient have other medical conditions?
- Is patient taking medications?
- When was patient's last tetanus shot?
- Has the patient completed a Rabies vaccination series previously?

Snakebites

- Type of snake? If safe to do so, take picture of snake and show hospital. Do not handle snake.
- Timing of bite?
- Location of bite on body?
- Is patient having significant pain or swelling to site, is erythema circumferential, around a limb, are there systemic symptoms (syncope, difficulty breathing, etc.)? – If so, this likely indicates significant envenomation.
- Does patient have other medical conditions?
- Medications – especially aspirin, Coumadin, other anticoagulants?

EMT/AEMT

- Follow General Medical Care Guideline
- Scene safety – Do Not handle snake or animal
- Primary assessment
- Remove jewelry and constricting clothing from affected extremity

Animal/Human Bite

- Wash area
- Control bleeding
- Dry bandage
- Secondary assessment
- See Rabies Guideline for reference

Snakebite

- Wash site with soap and water
- Immobilize area – do not elevate or use tourniquet
- Mark edge of pain (P), edge of tenderness (T), edge of skin changes (S) and note time
- 12-Lead EKG if indicated
- Secondary assessment, evaluate for signs of anaphylaxis and progression of PTS (mark progression with time)
- **Do not put ice on the wound, place a constricting device around the extremity, or attempt to suck out the venom**
- **All snakebites require evacuation**

Paramedic

Pain Control

- Per Pain Management Guideline

Human/Animal Bite

- Antibiotics - Ceftriaxone 2g IM or slow IV push daily OR Augmentin 875 every 12 hours for 3 days
- Must be evacuated if infection is established or wound repair is required - extensive tissue damage, over a joint, over a fracture site, face, hands, feet, genitalia

Rabies Consideration - See Rabies Guideline

- Rabies is 100% fatal and therefore worth preventing
- The closer to the brain or highly innervated region the bite, the more likely and faster the spread to the brain
- Exposure: Bite or saliva-contaminated scratch, open wound or mucous membrane exposure to saliva, CSF or central nervous system tissue from an animal with rabies
- Consider evacuation for rabies prophylaxis in people with following exposures:
 - Anyone who wakes up with a bat in an enclosed space (house or tent)
 - Bites/exposure from high-risk wild animal: bat, raccoon, skunk, fox, coyote, bobcat, woodchuck
 - Bites/exposure from unvaccinated dog, cat, or ferret unavailable for a 10 day quarantine
- Contact public health authorities for assistance the following exposures:
 - Bites/exposure from outdoor housed small pocket pet: guinea pig, rabbit, gerbil, hamster
 - Bites/exposure from livestock or unknown wild animal